# Case study: Natalie

Condition: **Spinal cord injury – T10 AIS A paraplegia**

44-year-old woman · Community occupational therapy home visit

Natalie Brooks is a 44-year-old woman with **T10 AIS A paraplegia** following a motor vehicle crash six years ago. She is a participant in the National Injury Insurance Scheme Queensland (NIISQ). Natalie is a single parent of two children, Zara (13) and Oscar (10), and lives with them in a lowset house in Logan. She works from home full-time as an accountant and drives a car fitted with hand controls.

Natalie has been well and independent since her rehabilitation. She manages her own bladder and bowel care, transfers independently and uses a manual wheelchair. The referral from her NIISQ support planner states that her pressure care cushion is "worn and bottoming out" and requests a new wheelchair and cushion prescription. No other concerns are listed.

You have been asked to complete a community home visit with a wheelchair supplier representative, who has brought a trial wheelchair and an air-cell cushion. Natalie greets you at the door, is organised and friendly, and has cleared a space in the lounge room. She tells you she would also like advice about her desk height and about her right shoulder, which has been aching when she pushes up the driveway. She has also printed a NIISQ equipment form that her support planner wants completed by Friday.

Natalie transfers neatly onto the trial wheelchair. When she moves back across to her own chair, you notice a damp, yellowish stain on the cover of her old cushion and an unpleasant smell. When you ask about it, Natalie hesitates, then says, "It's just a sore on my sitting bone. I've been dressing it myself with a mirror." She tells you it appeared about three weeks ago over her right buttock, after a run of long days at her desk. "It was tiny at first. It's bigger now, and it's been leaking a bit. I bought some dressings from the chemist."

As you talk, Natalie wraps a cardigan around herself although the room is warm. She says she has felt "flu-ish" for two days, had chills last night, and has woken up drenched in sweat. Her leg spasms have been "much worse than usual", and twice this morning her legs jumped off the footplate. She looks pale and tired. "I think I'm just run down. This time of year is brutal for accountants."

When you raise the possibility of seeing a doctor today, Natalie is firm. "I can't go to hospital. There's no one to mind the kids, and I've got a client deadline on Friday. I'll ring the GP next week." Her former partner lives interstate, and she says her mother "isn't well enough" to help.

There is no record of when Natalie last saw her GP, and no documentation of the wound in the referral or any recent nursing or medical review. You do not have a thermometer, and you are unsure whether anyone has looked at the wound apart from Natalie.

It is now 2.20 pm. The supplier representative explains that he needs to leave for his next appointment and asks whether you are happy to sign off on the trial wheelchair and cushion so he can send the quote through to NIISQ this week. Natalie checks the clock, reaches for her car keys and says, "School pick-up is at three and I have to leave by quarter to. Can we just get the cushion sorted?"

**You are the occupational therapy student completing a wheelchair and cushion prescription home visit with Natalie and your clinical supervisor is not currently in the room.**

**Present Natalie's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.**

# ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

## I Introduction

*≈ 20 s*

Who you are, your role, who the client is and why you need to talk now.

## S Situation

*≈ 40 s*

A brief but complete summary of what is happening and the immediate concern.

## B Background

*≈ 1 min*

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

## A Assessment

*≈ 2 min 20 s*

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

## R Recommendation

*≈ 2 min 40 s*

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

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